

HOW TO OPEN A GROUP HOME BUSINESS in South Carolina

Community Training Homes (CTH-I & CTH-II) and Supported Living under the ID/RD Waiver.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 • Every contact, rate & fee should be re-verified before you rely on it.

The South Carolina model — read this first

“Group home” in South Carolina means providing residential support to people with intellectual and related disabilities, and the state funds it mainly through the Intellectual Disability/Related Disabilities (ID/RD) Waiver, run by the disabilities agency historically known as DDSN. One recent change to know: as of April 2025, the agency formerly called the SC Department of Disabilities and Special Needs (DDSN) became the Office of Intellectual and Developmental Disabilities (OIDD) within the new Department of Behavioral Health and Developmental Disabilities (BHDD). You'll still see “DDSN” across most documents, forms, phone trees, and web addresses — it's the same system, so this guide writes it as BHDD-OIDD (DDSN). Two things make South Carolina distinctive, and they run through everything below. First, the residential support is delivered mostly through two “Community Training Home” models — CTH-I (the family model) and CTH-II (the staffed group home) — plus Supported Living (SLP). Second, homes are LICENSED annually, and getting paid means becoming a qualified BHDD-OIDD (DDSN) provider AND enrolling with SC Medicaid (SCDHHS).

The two ways in. Path A — become a CTH-I support provider: up to two people live in your OWN home and become part of your family, and you are a qualified, trained private citizen sponsored by a provider agency. Smallest, fastest, closest to a host home. Path B — run CTH-II group homes (or Supported Living) as your OWN provider agency: up to four people live in a home your agency operates, staffed and licensed, and you become a qualified BHDD-OIDD (DDSN) provider, enroll with SC Medicaid, and license each home. A much bigger lift — but it's the real, scalable business. This guide walks both, and is honest about which is realistic when.

The names you'll deal with. BHDD-OIDD (DDSN) = the Office of Intellectual and Developmental Disabilities (formerly the Dept. of Disabilities and Special Needs), within the Dept. of Behavioral Health and Developmental Disabilities — it sets standards, enrolls/qualifies providers, and licenses homes. SCDHHS = the SC Department of Health and Human Services (Medicaid) — holds administrative authority over the waivers and publishes the fee schedule. DHEC = the health/environmental agency involved in some licensing/health reviews. The waivers = ID/RD, Community Supports (CS), and HASCI — Residential Habilitation is an ID/RD service. Local DSN board = your county's Disabilities and Special Needs board, the local planning/coordinating entity (and often a provider and a CTH-I sponsor). Case Manager = develops and coordinates the person's plan (conflict-free case management is required).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change, so verify each step with BHDD-OIDD (DDSN), SCDHHS, your local DSN board, your fire authority, and qualified local counsel (see Part 12) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start as a CTH-I in your own home or build your own agency.

1	Decide your path & set up the business	Path A (CTH-I) or Path B (CTH-II/SLP agency); entity, EIN, NPI, insurance (Parts 3–4)
2	Learn the DDSN standards	Read the Residential Habilitation Standards and the Residential Licensing Standards (Part 5)
3	Become a BHDD-OIDD (DDSN) provider	Apply through provider enrollment — (803) 737-4917 (Part 5)
4	Enroll with SC Medicaid (SCDHHS)	Become an SC Medicaid provider so you can bill the waiver (Part 5)
5	License each home	Annual residential licensing by DDSN's quality organization, with fire & DHEC (Part 6)
6	Ready the home	Fire safety, HCBS settings rights, capacity by model (Part 6)
7	Staff & write your policies	Direct-support staff, RN oversight, documentation (Parts 7–8)
8	Get a person placed	Eligibility, waiver slot, the plan, freedom of choice & the match (Part 9)
9	Deliver services & get paid	Bill SC Medicaid against the authorized plan at the fee-schedule rate (Part 10)

Rule of thumb. Start smaller than you think. The fastest, lowest-cost way in is a CTH-I in your own home under a sponsoring provider or your local DSN board — you don't build a staff, and the sponsor handles training and billing. Many people begin as a CTH-I support provider, learn the system, and grow into their own CTH-II agency once they know the terrain.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by Medicaid, not a real-estate play or a normal small business: your revenue depends on becoming a qualified provider, enrolling with SC Medicaid, and licensing your home — and then on a person choosing you and having their services authorized, so your reputation with BHDD-OIDD (DDSN), your local DSN board, Case Managers, and established providers is your most valuable asset. Second, the money arrives on a delay: becoming a provider, enrolling with Medicaid, and licensing a home take months, and you don't bill until a person is placed and their services are authorized on the plan — so you need patience and a working-capital cushion to carry any staff and setup costs through the ramp. Start on Path A as a CTH-I support provider under an established sponsor if you can, learn the system on someone else's provider agreement, and grow into your own CTH-II agency once you know the terrain.

Here's how the rest of this guide is organized: Parts 1–2 explain what the business is and who regulates and pays you; Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks the two paths in — CTH-I and CTH-II; Part 5 is the provider-enrollment-and-Medicaid chain; Part 6 covers the home, licensing, and fire safety; Parts 7–8 are staffing and the policies and records DDSN licenses; Part 9 is how a person actually gets placed with you; Part 10 is how you get paid; Part 11 covers costs, timeline, and getting placements; and Part 12 collects every phone number and script in one place. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing residential support to people with intellectual and related disabilities under the ID/RD Waiver — as a CTH-I family home (up to two people), a CTH-II group home (up to four), or Supported Living. Paid by SC Medicaid, after BHDD-OIDD (DDSN) qualifies you and licenses the home.*

A “group home” business in South Carolina is really a developmental-disabilities support business. You help a person with an intellectual or related disability live successfully in the community — with daily support, supervision, skill-building, help with health and medications, and a real home life — rather than in an institution. South Carolina's main residential waiver service for this is Residential Habilitation, delivered through the two Community Training Home models and Supported Living. A Community Training Home-I (CTH-I) is the family model: up to two people live in a support provider's own home and essentially become part of the family. A Community Training Home-II (CTH-II) is the group-home model: up to four people live in a home operated by a provider agency, under qualified, trained staff. Supported Living (SLP) supports people living more independently in their own homes or apartments.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded by Medicaid — not a real-estate play. Your value is the quality and reliability of the support you provide and your ability to keep it compliant, licensed, and well-documented, because that's what BHDD-OIDD (DDSN) qualifies and licenses, what SC Medicaid pays for, and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based services for people with intellectual and related disabilities is high and durable: South Carolina, like every state, moves people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and the ID/RD Waiver funds exactly this. Because the person chooses their provider (freedom of choice), a reliable, well-run provider that builds relationships with local DSN boards and Case Managers can keep its homes full. And you can start lean — as a CTH-I supporting one or two people in your own home under an established sponsor — and grow into your own CTH-II agency as you learn the system.

Be honest about the two realities that trip up new providers. (1) *Licensing and qualification add steps: you don't just get a business license and bill Medicaid — you become a qualified BHDD-OIDD (DDSN) provider, enroll with SCDHHS, AND get each home licensed annually before a dollar flows.* (2) *Room and board is separate from your pay: the waiver pays for the SERVICE, not the person's rent and food, which come from their own income (usually SSI).* Plan your business — and explain it to families — around both.

PART 2

Who Regulates You & Who Pays You

In short: *Two bodies stand between you and a paid claim: BHDD-OIDD (DDSN) qualifies you as a provider and licenses your home, and SCDHHS (Medicaid) enrolls you and pays for services. Your local DSN board is your closest partner, and the Case Manager builds each person's plan.*

BHDD-OIDD (DDSN) — your qualifier and licenser

BHDD-OIDD — the Office of Intellectual and Developmental Disabilities, formerly the Department of Disabilities and Special Needs (DDSN), now within the Department of Behavioral Health and Developmental Disabilities (BHDD) — sets the standards, enrolls and qualifies providers, and licenses residential settings. Without becoming a qualified BHDD-OIDD (DDSN) provider you cannot deliver CTH/SLP services. Provider enrollment is (803) 737-4917 (or write the Intellectual Disability Division, P.O. Box 4706, Columbia, SC 29240); the eligibility/intake front door is 1-800-289-7012; the Adult Supports Office is (803) 898-9671.

SCDHHS — SC Medicaid (your enroller and payer)

After (and alongside) becoming a qualified DDSN provider, you enroll as a South Carolina Medicaid provider through SCDHHS so you can bill for waiver services. SCDHHS holds administrative authority over the ID/RD, CS, and HASCI waivers and publishes the HCBS Waiver Fee Schedule — the document that sets your rates. Residential Habilitation (CTH-I and CTH-II) is billed to SC Medicaid, and the rates come from that fee schedule.

Your local DSN board & the Case Manager

Your county's local Disabilities and Special Needs (DSN) board is the local planning and coordinating entity — and often a provider and a CTH-I sponsor. It's your most important local relationship: it can sponsor a CTH-I home, tell you the current need, and connect you into the system. The Case Manager develops and coordinates each person's support plan (conflict-free case management is required), identifies the residential model (CTH-I or CTH-II), and — with the person's choice — matches them to your home.

What this means for you: the path to payment is a chain — BHDD-OIDD (DDSN) qualifies you, SCDHHS enrolls you, each home is licensed, and then a Case Manager's plan authorizes the service — and each link takes time. Start them in the right order and in parallel where you can, and treat your local DSN board and area Case Managers as key relationships, because their people choose their providers. A home that's qualified, enrolled, licensed, and known to Case Managers is a home that fills.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In South Carolina you form the LLC through the Secretary of State. Your entity, EIN, and NPI all have to be in place and consistent before you become a qualified DDSN provider and enroll with SC Medicaid, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the SC Secretary of State.** Online at sos.sc.gov — the LLC filing fee is about \$110. Name the LLC and list a registered agent with a physical South Carolina address (this can be you).
- 2. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your DDSN provider file, and your Medicaid enrollment will reference your structure.
- 3. No annual report for an LLC.** South Carolina LLCs do not file an annual report with the Secretary of State (that's a corporation requirement) — a real simplification. Confirm your tax registrations with the SC Department of Revenue.
- 4. Confirm local business licenses.** Many SC cities and counties require a local business license — check what applies where you'll operate.

No newspaper publication in South Carolina. South Carolina has no LLC publication requirement — forming the entity is a straightforward Secretary of State filing (about \$110), with no publication and no LLC annual report.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with SCDHHS as an SC Medicaid provider tied to your BHDD-OIDD (DDSN) provider record, and to bill SC Medicaid for CTH/SLP services. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a developmental-disabilities / community-based residential provider — confirm the exact one SCDHHS (SC Medicaid) and DDSN billing want. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of South Carolina branches — South State Bank, First Citizens (Columbia-based), Synovus, TD Bank, Wells Fargo, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A CTH/SLP provider brings staff into people's homes (or opens their own home) and supports vulnerable people, so you need coverage built for a human-services provider — and BHDD-OIDD (DDSN) and SC Medicaid will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the SUPPORT and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a person you support — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended for an agency
Non-Owned & Hired Auto	Staff driving their own cars to support or transport the people you serve	Yes — critical
Workers' Compensation	Direct-support staff injuries	Required in SC (4+ employees)
Property	Coverage on any provider-owned or leased home	If you own/lease a home

Abuse & molestation is your highest exposure. Supporting vulnerable people means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD services include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local South Carolina agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for a developmental-disabilities / CTH provider program. Confirm the carrier writes this in South Carolina before you rely on a quote.

What to ask the insurance agent

“Do you write developmental-disabilities / CTH-II / supported-living providers in South Carolina — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include non-owned & hired auto for my staff driving the people we support?”

“What limits do BHDD-OIDD (DDSN) and SC Medicaid expect?”

“Can you set up South Carolina workers' comp for my direct-support staff?”

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves South Carolina providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your BHDD-OIDD (DDSN) provider application, your SCDHHS Medicaid enrollment, and every home-licensing file. Your information passes through more than one organization before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there — is one of the most common and most frustrating causes of a stalled enrollment or a rejected claim. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front to confirm the structure, because untangling a mismatch after you've applied costs far more than getting it consistent now. And keep your provider status, Medicaid enrollment, and each home's annual license calendared for renewal — letting any one of them lapse stops your payments even when the care is going perfectly.

The business foundation to have in hand before you apply to become a provider:

- Your formed SC LLC (Articles of Organization filed with the Secretary of State) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance — general/professional liability, abuse & molestation, non-owned auto, and workers' comp
- A single, consistent legal name, ownership list, and address used identically on every document

With those six things in place and consistent, you're ready to build the provider application in Part 5 on a clean foundation — and you've removed the paperwork mismatches that stall more South Carolina applications than any substantive problem does.

PART 4

The Two Paths — CTH-I (A) or CTH-II / SLP Agency (B)

In short: Path A is the fast, small way in — become a CTH-I support provider in your own home under a sponsoring provider. Path B is the real business — run CTH-II group homes (or Supported Living) as your own qualified agency. Pick the one that fits where you are today; many start with A and grow into B.

4.1 Path A — a Community Training Home-I (CTH-I) in your own home

A CTH-I means up to two adults with intellectual or related disabilities live in your home and become part of your family. You're a qualified, trained private citizen sponsored by a BHDD-OIDD (DDSN) provider (often your local DSN board or a private agency), and the sponsor recruits, trains, approves, and supports you and handles the billing. It's the fastest way in and the closest thing to a host home — and you don't build a staff.

- 1. Make sure it fits your life.** Up to two people will live in your home and become part of your family, so everyone in the household is part of this — it's a family decision. Go in with eyes open about the good days and the hard ones.
- 2. Connect with a sponsoring provider or your DSN board.** CTH-I support providers are sponsored by a DDSN provider. Ask your local DSN board who sponsors CTH-I homes in your area and what the current need is — and talk to more than one sponsor.
- 3. Get approved, trained, and licensed.** The sponsor completes your approval and the required training, and your home is licensed by DDSN's quality organization. A CTH-I follows the State Fire Marshal's Foster Home Regulations — lighter than a commercial facility, because it's a private family home.
- 4. Pass background checks, then get matched.** You and the adults in your home complete the required criminal-background and abuse-registry checks; then the provider, the person's Case Manager, and the team match you with someone whose needs and personality fit your home — and the person has freedom to choose.

***The difficulty-of-care tax note.** Because the person you support lives in your OWN home, your CTH-I payments may qualify for the IRS "difficulty-of-care" exclusion (potentially non-taxable). Confirm with a tax professional — it can meaningfully change the economics of a CTH-I arrangement.*

4.2 Path B — run CTH-II group homes (or Supported Living) as your own agency

Path B is more work, more money, and more control: you become a qualified BHDD-OIDD (DDSN) provider, enroll with SC Medicaid, and license and operate CTH-II group homes (up to four residents each, staffed around the clock) or Supported Living. It's the real, scalable business — and Part 5 walks the provider path step by step. Most operators who go this route either have direct experience in DD services or hire someone who does, because your application has to show DDSN you understand the Residential Habilitation Standards and can deliver.

PART 5

Become a BHDD-OIDD (DDSN) Provider & Enroll with SC Medicaid

In short: For Path B, this is the chain that unlocks payment: learn the standards, apply to become a qualified DDSN provider, enroll with SCDHHS as an SC Medicaid provider, then license each home before anyone moves in.

- 1. Learn the standards cold.** Read the BHDD-OIDD (DDSN) Residential Habilitation Standards, the Residential Licensing Standards, and the ID/RD Waiver manual closely — your application must show you understand them and can deliver services to the standard.
- 2. Apply to become a qualified DDSN provider.** Contact BHDD-OIDD (DDSN) provider enrollment — (803) 737-4917, or write the Intellectual Disability Division, P.O. Box 4706, Columbia, SC 29240 — and apply online. You'll meet the Residential Habilitation Standards and the applicable DDSN directives and standards.
- 3. Enroll with SC Medicaid (SCDHHS).** Complete South Carolina Medicaid provider enrollment through SCDHHS so you can bill for waiver services. SCDHHS holds administrative authority over the waivers and publishes the HCBS Waiver Fee Schedule (your rates).
- 4. License each home.** Every residential setting must be licensed. DDSN's quality organization (QIO) licenses CTH-II (and CTH-I and SLP-II) homes annually under the Residential Licensing Standards; reviews can involve DDSN, DHEC, and the State Fire Marshal. Get the home licensed BEFORE anyone moves in.
- 5. Put policies and staffing in place, then serve.** Put your policies and procedures in place (Part 8), hire and train your direct-support staff, and make sure each resident has the required lease/residency agreement. Then accept referrals, deliver services, and bill SC Medicaid (Parts 9–10).

What this means for you: becoming a provider is a quality gate, not a form. DDSN wants evidence — real policies, real service capacity, a licensable home — that you can safely support people. The providers who get through cleanly treat the Residential Habilitation and Licensing Standards as the blueprint for the whole operation, so building your policies, staffing, and home to the standard does double duty: it qualifies you AND it's how you'll actually run.

PART 6

The Home — Licensing, HCBS Rights & Fire Safety

In short: Every residential setting is licensed annually by DDSN's quality organization under the Residential Licensing Standards. A CTH-I (family model) follows the State Fire Marshal's lighter Foster Home Regulations; a CTH-II carries fuller fire and licensing review. In every case, it must genuinely be the person's home.

6.1 How South Carolina handles the home

Every residential setting is licensed. BHDD-OIDD (DDSN)'s quality organization licenses CTH-I, CTH-II, and SLP-II homes annually using the Residential Licensing Standards, and reviews can involve DDSN, DHEC, and the State Fire Marshal. A CTH-I (family model) specifically follows the State Fire Marshal's Foster Home Regulations — lighter than a commercial facility, because it's a private family home. In every case, the home must genuinely be the person's home — with a lease or legally enforceable residency agreement, privacy, and full access to the community.

6.2 “Do I need fire sprinklers?” — the honest answer

For a CTH-I (up to two people in your own home), the home is held to the State Fire Marshal's Foster Home Regulations — a residential standard — so a full commercial sprinkler system generally is NOT required. For a CTH-II (up to four residents in a provider-operated home), fire requirements come through the annual licensing review and the State Fire Marshal, and depend on the home's size and construction — so a staffed group home may face requirements a family home doesn't. Because it's set by the fire code and the State Fire Marshal, confirm your specific home with them and with DDSN BEFORE you buy or renovate.

6.3 The home checklist

- ☐ Annual licensing by DDSN's quality organization (Residential Licensing Standards) — health and safety, staff ratios, fire safety, medication management, facility size and construction, and hazardous-materials storage.
- ☐ A fire-safety review — CTH-I under the State Fire Marshal's Foster Home Regulations; CTH-II under the licensing review and the State Fire Marshal.
- ☐ Working smoke and carbon-monoxide detectors, extinguishers, and clear exits with a practiced evacuation plan.
- ☐ HCBS settings rights: each resident has a lease or legally enforceable agreement, privacy in their unit (including lockable doors), choice of roommates, and the freedom to furnish and decorate.
- ☐ Documentation kept on-site at each residential location — licensing reviews check it.
- ☐ Capacity respected by model — a CTH-I serves up to two people; a CTH-II serves up to four.

Two things people get wrong. (1) Capacity is defined by the model — a CTH-I serves up to two people; a CTH-II serves up to four. Plan the model to the home and the people you'll serve. (2) Room and board is separate from your pay — the service rate doesn't cover the person's rent and food; those come from their own income (usually SSI). Keep the two completely separate in your books and your agreements.

PART 7

Who You Actually Need — Staffing

In short: As a CTH-I (Path A), you and your household provide the support, with the sponsoring provider's oversight, training, and billing — you don't build a staff. As a CTH-II agency (Path B), plan for a director, direct-support staff, nurse oversight, quality/compliance, and billing — one person can wear several hats at the start.

7.1 The roles

CTH-I (Path A): you and your household are the support. The sponsoring provider gives you the professional oversight, training, and billing, and helps arrange respite. You don't build a staff. CTH-II agency (Path B), plan for these roles:

- **Agency director / administrator:** runs the agency and owns the BHDD-OIDD (DDSN) provider agreement and the home licensing.
- **Direct-support staff:** the qualified, trained caregivers in your CTH-II homes, staffed to meet residents' needs around the clock.
- **Registered Nurse:** for health oversight and medication management — often part-time or contracted at first.
- **Quality / compliance:** to keep up with the Residential Licensing Standards and prepare for the annual licensing reviews.
- **Billing / administrative support:** handles SC Medicaid billing and documentation (kept on-site at each home).

7.2 Recruiting and keeping direct-support staff — the real make-or-break

Direct-support staff are the heart of a CTH-II business and the hardest thing to keep. Turnover is high across the whole field, and a home that can't reliably staff its shifts can't safely support residents or keep its license — so treat recruiting and retention as your core operation, not an afterthought.

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability.
- **Onboard to the standard:** real orientation, the required background and abuse-registry checks and training, and a competency check before a staff member supports someone alone — this protects residents and satisfies DDSN licensing.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a staff member hits a hard day, and recognition. Every person who quits costs you re-hiring, re-training, and often continuity for the resident they supported.

PART 8

Policies, Records & Staying Compliant

In short: *Your policies and documentation are what DDSN licenses and what SC Medicaid pays against. Build them to the Residential Habilitation and Licensing Standards from day one — medications, emergencies, incident reporting, and the rights of the people you support.*

You keep clear documentation covering medications, emergencies, incident reporting, and the rights of the person you support — including their lease/residency protections — and the person's support plan drives what you deliver and record. This documentation is both the DDSN licensing requirement and the backbone of a safe, well-run operation. Build a simple system that keeps each person's plan, service records, and incident documentation current and on-site, because DDSN's annual licensing review checks it and SC Medicaid can audit it.

8.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 9

The Person's Journey — Eligibility to Move-In

In short: *You can't get paid until someone is placed with you and their services are authorized on the plan. The path runs from eligibility and a waiver slot, through the Case Manager's plan, to freedom-of-choice and the match.*

- 1. The person qualifies — and often waits.** They have a developmental disability apparent before age 22, meet an ICF/IID level of care, and are Medicaid-eligible. BHDD-OIDD (DDSN) determines eligibility; intake often starts with a local DSN board or the toll-free line (1-800-289-7012). Slots are limited, so people request a slot and may wait — a real feature of the system.
- 2. They get a waiver slot.** Residential Habilitation is funded through the ID/RD Waiver; people move from requesting a slot into an available slot on the state's schedule.
- 3. The plan is built.** A Case Manager develops the support plan (conflict-free case management), which identifies the residential model — CTH-I or CTH-II — and the person's needs.
- 4. The match.** The person chooses providers (freedom of choice); you (or your sponsoring provider/agency) are matched based on fit and your licensed services.
- 5. Move-in.** With the plan and a licensed home in place, the person moves in and you're delivering a billable service.

What this means for you: because people choose their provider and slots are limited, your census depends on relationships and reputation, not marketing. Get known to your local DSN board, to established providers, and above all to the Case Managers in your area as the provider who supports people well, communicates, and handles a placement smoothly — and the matches follow.

Plan your business around the waitlist reality. *The waiting list for waiver slots is real, and slots open on their own schedule — which shapes how you should plan. Don't build a business model that assumes you'll fill four beds the month you're licensed; assume a gradual build as Case Managers learn to trust you and matches come through. Two practical moves help: start on Path A as a CTH-I support provider under an established sponsor so you have revenue while you build, and stay in regular, helpful contact with the Case Managers and DSN board in your area so that when one of their people is choosing a provider, you're the name they think of first. Patience plus reliability is the whole strategy — the demand is real, but it arrives one authorized person at a time.*

PART 10

How You Get Paid: The Fee Schedule & Your First Payment

In short: Residential Habilitation (CTH-I and CTH-II) is billed to SC Medicaid at rates set by SCDHHS in the HCBS Waiver Fee Schedule. The service is authorized on the person's plan; you bill for what you deliver. Room and board is separate.

10.1 How the rate works

- **Set by SCDHHS:** current codes and rates are in the SCDHHS HCBS Waiver Fee Schedule ([scdhhs.gov](https://www.scdhhs.gov) → Fee Schedules) — look up Residential Habilitation for CTH-I and CTH-II. Confirm the current figure before you budget.
- **CTH-I vs. CTH-II:** the two models are priced separately — a CTH-II (staffed, provider-operated) carries different costs and a different rate than a CTH-I (family model).
- **Billed to SC Medicaid:** for a CTH-I, the sponsoring provider files the claims; a CTH-II agency bills directly. The service is authorized on the person's plan, and you bill Medicaid for the service you deliver.
- **Room and board is separate:** Medicaid pays the service, not the person's rent and food (those come from their SSI/personal income). Never bill the person for a covered service.

10.2 The money math — how a CTH-II pencils out

CTH-II economics are different from an hourly private-pay business — your revenue is the fee-schedule rate for the authorized Residential Habilitation service, and your biggest cost is direct-support labor. Lay it out with your own numbers:

- **Revenue:** the authorized Residential Habilitation service on each resident's plan, billed to SC Medicaid at the CTH-II fee-schedule rate — up to four residents in a home.
- **Cost:** direct-support wages and payroll taxes are the bulk; then RN oversight, administration, insurance, licensing, and the building. Most are fixed against your licensed capacity.
- **The lever:** because the rate is set by the fee schedule, your margin comes from staffing efficiently to residents' authorized needs and keeping your licensed home filled with authorized people — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the resident's rent and food come from their own income (usually SSI), NOT the waiver — keep that money and those agreements entirely separate from your service billing.

10.3 Your first payment

1. **Confirm the service is authorized on the person's plan.** The right model (CTH-I or CTH-II) and level must be on the plan before you bill.
2. **Deliver the service and keep your documentation on-site.**
3. **Submit the claim to SC Medicaid.** The sponsoring provider does this for a CTH-I; a CTH-II agency bills directly.
4. **Get paid — and bill only for services actually provided.** Keep your provider status, Medicaid enrollment, and home license current so payment never lapses.

PART 11

What It Costs, How Long It Takes & Getting Placements

In short: A CTH-I (Path A) can start relatively quickly; a CTH-II agency (Path B) takes several months through provider enrollment, Medicaid enrollment, and home licensing. Placements come through freedom of choice, so your relationships with Case Managers and your DSN board are your census.

11.1 Timeline & startup costs

A CTH-I (Path A) can move relatively quickly once you've connected with a sponsoring provider — approval, training, background checks, and licensing of your home. A CTH-II agency (Path B) takes longer: the business setup, becoming a BHDD-OIDD (DDSN) provider, SC Medicaid enrollment, and licensing each home — plan for several months end to end. CTH-I costs are modest — mainly your time, background checks, training, and getting your home ready for the Foster Home fire review and licensing. A CTH-II agency adds business formation, insurance, staff payroll, the annual licensing of each home, and your policies and procedures. Confirm current fees and timelines with BHDD-OIDD (DDSN), SCDHHS, and your local DSN board.

11.2 Getting your first placements

Placements don't come from advertising — they come through freedom of choice and your reputation. Here's a simple plan:

- 1. Get known to your DSN board and established providers.** Call your local DSN board and BHDD-OIDD (DDSN) provider enrollment (803) 737-4917, and — if you're starting on Path A — connect with the providers who sponsor CTH-I homes in your area.
- 2. Build relationships with Case Managers.** They develop the plan and, with the person's choice, match people to providers. Be the provider they trust to handle a placement well, communicate, and never leave a shift unstaffed.
- 3. Be responsive and reliable.** Answer the phone, say yes to a good match, handle move-in smoothly, and follow through — one good placement earns the next referral.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: BHDD-OIDD (DDSN) and your local DSN board are the center of gravity, with the State Fire Marshal and SCDHHS close behind. Statewide contacts are the same for everyone; your fire/building offices are local. Here's the list and the scripts.

12.1 Statewide (same for everyone)

For...	Contact
Becoming a provider	BHDD-OIDD (DDSN) provider enrollment — (803) 737-4917 · ID Division, P.O. Box 4706, Columbia, SC 29240
Applying / eligibility / intake	BHDD-OIDD (DDSN) — 1-800-289-7012 · or your local DSN board
Adult supports	DDSN / BHDD-OIDD Adult Supports Office — (803) 898-9671
Rates (fee schedule)	SCDHHS — scdhhs.gov → Fee Schedules (HCBS Waiver Fee Schedule)
Home licensing & fire	BHDD-OIDD (DDSN) Quality/Licensing · State Fire Marshal (Foster Home Regulations for CTH-I) · DHEC where relevant
Business, EIN & NPI	sos.sc.gov · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

12.2 The local calls — and exactly what to ask

① Your local DSN board / a sponsoring provider — your most important calls

"I want to open a CTH-I in my home / a CTH-II group home. Who sponsors CTH-I homes here, and what's the current need?"

"(To a sponsor) How do you approve, train, and support CTH-I providers, how do you match, and how and when do you pay?"

② BHDD-OIDD (DDSN) Licensing — the annual review

"What do the Residential Licensing Standards check for a CTH-I / CTH-II, and can I get the standards to self-assess before I open?"

③ State Fire Marshal / local fire authority — the fire review

"I'm opening a CTH-I (foster-home model) / a CTH-II for up to four residents. What fire-safety requirements apply, and do I need sprinklers?"

12.3 Worked examples — South Carolina's two biggest markets

BHDD-OIDD (DDSN) intake and the provider process are statewide — the local piece is mainly your DSN board and your fire authority. South Carolina has a local DSN board in every county; find yours by where you live.

- **Columbia (Richland / Lexington):** Richland/Lexington Disabilities and Special Needs Board (a statewide-approved provider serving the Columbia metro); BHDD-OIDD (DDSN) headquarters is in Columbia, with the Adult Supports Office (803) 898-9671 and provider enrollment (803) 737-4917; DDSN Licensing for the annual review, and the State Fire Marshal / City of Columbia (or Richland/Lexington County) fire authority for the fire review. The quirk: Columbia is the capital and a busy market — the Richland/Lexington board and several private providers sponsor CTH-I homes.
- **Greenville (Upstate):** Greenville County Disabilities and Special Needs Board (P.O. Box 17467, Greenville, SC 29606); the same statewide BHDD-OIDD (DDSN) process and provider enrollment (803) 737-4917; DDSN Licensing, and the State Fire Marshal / City of Greenville (or Greenville County) fire authority. The quirk: Greenville is South Carolina's largest metro — strong demand, and multiple Upstate providers sponsor CTH-I homes.

How to find your own county's board & offices: find your local DSN board (there's one for every county) via ddsn.sc.gov → provider directory, or call 1-800-289-7012; start the provider process with BHDD-OIDD (DDSN) provider enrollment at (803) 737-4917; and for fire safety contact the State Fire Marshal or your city/county fire authority, while for licensing you work with DDSN Quality.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to become a licensed agency, or can I start smaller?

You can start smaller — Path A lets you become a CTH-I support provider in your own home, sponsored by an established DDSN provider or your local DSN board that handles training and billing. It's the fastest way in and the best way to learn the system before you take on your own DDSN provider status and home licensing (Path B). Many providers grow from A to B.

Why do I see “DDSN” everywhere if the agency changed?

As of April 2025 the Department of Disabilities and Special Needs (DDSN) became the Office of Intellectual and Developmental Disabilities (OIDD) within the new Department of Behavioral Health and Developmental Disabilities (BHDD). It's the same system — most documents, forms, phone trees, and web addresses (like ddsn.sc.gov) still say “DDSN,” so treat them as the same agency.

Does the waiver pay for the person's rent and food?

No — the waiver pays for the SERVICE (the support you provide), not room and board. The person's rent and food come from their own income, usually SSI. Keep the two completely separate in your agreements and your books; mixing them is a common and serious mistake.

What's the difference between a CTH-I and a CTH-II?

A CTH-I is the family model — up to two people live in your own home and become part of your family, and you're a qualified private citizen sponsored by a provider. A CTH-II is the group-home model — up to four people live in a home your agency operates, staffed and licensed. CTH-I is the smaller, faster start; CTH-II is the scalable business. Both are Residential Habilitation and both are licensed.

You can do this

Opening a group home business in South Carolina is real work — the licensing and qualification add a few steps — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Whether you start with a family-model CTH-I or build your own staffed CTH-II agency, take it one step at a time: set up the business, learn the DDSN standards, become a qualified provider and enroll with SC Medicaid, license the home, ready it, and build the relationships that bring you placements. Verify the current details with the State as you go, and lean on your local DSN board and BHDD-OIDD (DDSN) — that's what they're there for.

Reminder. Prepared for PolicyReady.org and grounded in South Carolina BHDD-OIDD (formerly DDSN) and SCDHHS rules and provider standards as of 2026 — including S.C. Code 44-20-10 et seq. (the DDSN/BHDD-OIDD enabling law) and Act No. 3 of 2025 (creation of BHDD), the Residential Habilitation and Residential Licensing Standards (CTH-I, CTH-II, SLP-II), the State Fire Marshal Foster Home Regulations, and the SCDHHS HCBS Waiver Fee Schedule and ID/RD Waiver manual. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with the State of South Carolina before you act.